DEPARTMENT OF VETERANS AFFAIRS (VA)

REPORT TO CONGRESS ON PRECISION ONCOLOGY PROGRAM FOR CANCER OF THE PROSTATE

Report Language: The agreement is disappointed that the Department has not yet expanded the number of POPCaP sites as required by the joint explanatory statement accompanying Public Law 118–42. As such, the agreement directs the Department to submit a report within 30 days after the enactment of this act detailing how it will maintain the 21 current POPCaP sites and expand to additional locations. Additionally, the agreement directs VA to provide a spend plan for how it allocated funding to the current sites in fiscal years 2024 and 2025, including how it spent the additional funds provided in the joint explanatory statement accompanying Public Law 118–42 specifically for the expansion of POPCaP, as directed, as well as a spend plan for maintaining and expanding POPCaP sites in fiscal year 2026 and beyond. *Senate Report 119-43, page 47.*

Discussion:

This report comprises four sections:

- **I.** Overview
- **II.** Cancer Clinical Research Centers (Hub and Spoke model) for sustainability and long-term impact
- **III.** Congressional Support and Expansion Initiatives
- **IV.** Conclusion

I. Overview

Over 50,000 Veterans are diagnosed with cancer annually at VA, and of these, approximately 15,000 have prostate cancer. Prostate cancer is the most common non-skin cancer among Veterans receiving care through VA. Since fiscal year (FY) 2018 the Prostate Cancer Foundation (PCF) established 14 Precision Oncology Program for Cancer of the Prostate (POPCaP) sites through its partnership with VA. Subsequently, the VA Office of Research and Development (ORD), in collaboration with the VA National Oncology Program Office, expanded the POPCaP network by adding 7 VA-funded genitourinary (GU) sites in FY 2021. This expansion primarily targeted the Midwest and Southeast regions to ensure geographic balance and address the higher Veteran population in these areas. As a result, the total number of POPCaP/GU sites increased to 21(see Appendix1). Currently all 21 POPCaP/GU sites continue to operate, providing patient care, actively participating in clinical trials, and conducting high impact research to improve the health and well-being of Veterans with prostate and other genitourinary cancers.

II. Cancer Clinical Research Centers (Hub and Spoke model) for sustainability and long-term impact

In FY 2024, as funding for the VA-PCF partnership was expiring, ORD in partnership with the National Oncology Program were in the process of implementing plans to meet the requirements of the Cleland-Dole Act P.L. 117-328 §102(c)(4) to increase focus on prostate cancer (see Congressionally Mandated Report: Prostate Cancer Research Funding Plan submitted to Congress in August 2023). The plans include a new structure and enterprise-wide strategy involving the use of a cancer clinical-research hub and spoke model in each of the 18 Veterans Integrated Service Network (VISNs) to support oncology care, clinical trials and research. A similar approach was used for increasing lung cancer care, screening, clinical trials and research in the Lung Precision Oncology Program. These hubs, endowed with specific areas of expertise including prostate cancer, will engage more Veterans and VA medical centers (VAMCs) over time while exponentially increasing the number of VA facilities/locations engaged in oncology, and collaboratively enhance clinical capabilities and research activities. The new model facilitates expanded partnerships between VAMCs within their VISNs and NCI-designated Cancer Centers, enhancing the opportunity for increased clinical trials and multidisciplinary research. This structural enhancement aims to bring advanced oncology care closer to Veterans, particularly those in rural and less urban areas, through the National Teleoncology platform (NTO).

The reorganization integrates all current POPCaP/GU sites as either hub or spoke sites into a coordinated overarching structure. This reorganization synergizes and maximizes investments in the platform through which success and best practices in prostate cancer can serve as a model for all cancers across the VA enterprise, as recommended by the Cleland-Dole Act. These changes will reduce duplicative efforts, streamline processes, coordinates resources to meet the real-world needs of Veterans with cancer, and support long-term sustainability. The nation-wide network of hubs and spokes will be coordinated by the Boston Cooperative Studies Program Coordinating Center. For years this organization's development of frameworks for centralized administration, operations, management, and dissemination of best practices has produced quality and novel scientific approaches to multi-site trials, epidemiological, and other research studies. An executive committee with representatives drawn from relevant program and operational offices (including ORD and National Oncology) will provide governance and oversight.

The transition to the VISN-wide hub-and-spoke model is designed to be sustainable, ensuring continuity of the 21 POPCaP/GU sites in clinical care, trials, and research despite the funding transition from PCF to VA. By integrating POPCaP/GU sites into this model, ORD aims to maintain a robust portfolio of prostate cancer research and expand capabilities to address other cancer types, thereby meeting the goals stipulated by the Cleland-Dole Act and support ongoing precision oncology initiatives contingent on continued annual congressional appropriations in future years. Additional resources to ensure high-quality clinical care will be provided by the National Oncology Program.

III. Congressional Support and Expansion Initiatives

Late in the third quarter of FY 2024, Congress provided $5 million to support expansion of POPCaP. ORD employs a competitive peer-review process using an open request for applications (RFA) to distribute research funding. This process ensures that funding is allocated based on scientific merit, identifying facilities with the necessary capabilities and expertise to function effectively and attain necessary research goals. The first set of 6 new cancer clinical research hubs and spoke sites were not selected until the end of FY 2024. This was due to the late timing of the budget and the time taken to complete the peer review process. The mission of the research hubs and spoke sites is to begin expanding the number of VA facilities focused on cancer care, clinical trials, and research. Distribution of funds to support the new sites and a coordinating center located at the Boston VA took place in early FY 2025. Additionally, sites that were not selected based on peer review, were provided start-up funds to support activities that would be a prerequisite to a successful center application in future competition and eventual integration in the expanded hub and spoke clinical research network. (see Appendix 2). It should be noted that many of these sites faced start-up delays due to challenges with hiring new staff, adhering to local facility personnel caps, and completing intergovernmental personnel act (IPA) agreements with NCI-affiliated medical centers to on-board technical and subject matter experts.

ORD issued a second solicitation for applications in the second quarter of FY 2025 to identify additional hubs and spoke sites working toward achieving nationwide coverage and ensuring all 18 VISNs access to high-quality oncology care and research as outlined in the plan. Seven additional hubs (and associated spoke sites) were chosen, which increased the total to 13 hubs. The 5 remaining sites that were not selected were asked to develop remediation plans to address gaps and weaknesses that included management/leadership experience, clinical capabilities, resource allocation, and lack of expertise found in their applications. Upon successful completion, review and approval of these remedial plans will take place in the second quarter of FY 2026 with the objective of being added to the clinical research network.

Together the hub and spoke sites will not only achieve the goals laid out in the “Prostate Cancer Research Funding Plan” submitted to Congress, but also significantly expand the number of sites focused on prostate (and other genitourinary) cancer care, clinical trials, and research. The current 13 hubs and spokes account for over 81 VA facilities across 13 VISNs and include a majority of the original POPCaP sites. Further they all have prostate and other genitourinary cancers (GU) as part of their immediate focus (see Appendix 3). Therefore, under this new integrated and comprehensive structure for oncology care, the number of VA facilities focused on prostate cancer will be quadrupled (that is, an increase from the original 21 to 81 sites). Once the remediation process is completed for the remaining 5 hubs and spoke, together they will add an additional 25-30 sites (for a total of 100 plus sites) to the clinical research network. This will enable systematic and equitable access to high-quality prostate cancer care, clinical trials, and research; and thereby meet the intent of the joint explanatory statement accompanying Public Law 118–42 for an increased number of VA facilities/locations focused on prostate cancer.

IV. <u>Conclusion</u>

The restructuring into cancer clinical research centers enabled by the Cleland-Dole Act will substantially benefit prostate cancer efforts within the VA, allowing for the replication of best practices to other cancer types and enhancing overall access to high-quality oncology care for Veterans. The integrated hub-and-spoke model ensures that current POPCaP sites will continue in the new cancer clinical research network and its work expanded to a significant number of VA facilities/locations. In addition, clinical care continuity will be maintained, emphasizing VA’s commitment to advancing prostate cancer research and treatment. Funding provided by Congress in FY 2024 supported the expansion of prostate cancer care, trials, and research at VA facilities, increasing the number of sites focused on prostate cancer with more than 100 sites in the new cancer clinical research center network.

Department of Veterans Affairs

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Appendix 1. List of POPCaP/GU sites

	VISN	Facility	City
PCF/POPCaP		VA Boston Healthcare System	Boston, MA
	2	James J. Peters VAMC	Bronx, NY
	2	NY Harbor HCS, incl. Manhattan and Brooklyn	Manhattan, NY
	4	Philadelphia VAMC	Philadelphia
	5	Washington DC VAMC	Washington DC
		*VA Maryland Healthcare System	Baltimore, MD
	6	Durham VAMC	Durham, NC
	8	Tampa VAMC/Bay Pines	Tampa/Bay Pines, FL
	10	VA Ann Arbor HCS	Ann Arbor, MI
	12	Jesse Brown VAMC	Chicago, IL
	20	VA Puget Sound HCS	Seattle, WA
	20	VA Portland Healthcare System	Portland, OR
	21	San Francisco VA Medical Center	San Francisco, CA
GU sites	7	Atlanta VAMC	Decatur, GA
	19	Rocky Mountain VAMC (Aurora/Denver)	Aurora, CO
	7	Ralph Johnson VAMC	Charleston, SC
	16	Michael DeBakey VAMC	Houston, TX
	15	Kansas City VAMC	Kansas City, MO
	15	St Louis VAMC	St Louis, MO
	8	Orlando VA Healthcare System	Orlando, FL

Note: * Denotes a satellite site within the same VISN.

Appendix 2. Spend plan (Attachment)

Appendix 3. List of 13 hubs (and spoke sites) in the Cancer Clinical Research Network

	Cancer Focus	VISN	Hub	City	Spoke sites
13 Selected hub sites	Genitourinary (GU, includes prostate, bladder, kidney)	1	**VA Boston VA (hub)***	Boston	Providence, Manchester, White River Junction, Togus
			VA Connecticut HCS (co-hub)	West Haven	
	Prostate/Lung	12	**WM S. Middleton Memorial Veterans Hospital**	Madison	Hines, Milwaukee, Chicago-Jesse Brown*, (Lovell FHCC, Tomah, Iron Mountain, Danville)
	Prostate/ hematological cancers	23	**Minneapolis VA (hub)**	Minneapolis	St. Cloud VA, Fargo VA
			VA Nebraska (co-hub)	Omaha	
			VA Iowa City (co-hub)	Iowa City	
	Prostate/Lung	4	**Philadelphia VA (hub)***	Philadelphia	Coatesville, Lebanon, Wilkes-Barre, Wilmington; Altoona, Butler, Erie
			VA Pittsburgh VAMC (co-hub)	Pittsburgh	
	Genitourinary (GU, includes prostate, bladder, kidney)	5	**Washington DC VA (hub)***	Washington, DC	Baltimore, Beckley, Clarksburg, Huntington, Martinsburg
	Genitourinary (GU, includes prostate, bladder, kidney); and hematological cancers	15	**Kansas City VAMC***	Kansas City	Topeka, Wichita, Leavenworth, Poplar Bluff, Marion, Columbia
			St Louis VAMC* (co-hub)		
	Genitourinary (GU, includes prostate, bladder, kidney), colon, esophageal, pancreas	10	**VA Ann Arbor Health Care System***	Ann Arbor	Detroit, Cleveland, Battle Creek, Sagninaw, Nothern Indiana, Dayton, Fort Wayne

			Indianapolis VA (co-hub)	Indianapolis	
	Genitourinary (GU, includes prostate, bladder, kidney), lung, colon, esophageal, pancreas	4	**VA New York Harbor Health Care System***	Manhattan	Northport, East Orange, Albany, Syracuse, Buffalo, Hudson Valley
			Bronx VA Medical Center (co-hub)*	Bronx	
	Genitourinary (GU, includes prostate, bladder, kidney), lung	21	**San Francisco VA Health Care System***	San Francisco	VA Northern California HCS (Sacramento), VA Central California HCS (Fresno), VA Southern Nevada HCS (Las Vegas), VA Sierra Nevada (Reno),
			VA Palo Alto Health Care System (Co-hub)	Palo Alto	
	Genitourinary (GU, includes prostate, bladder, kidney), hematological cancers	20	**VA Puget Sound Health Care System***	Seattle	Portland, Boise, Mann-Grandstaff, Walla Walla, White City, Anchorage, Roseburg
	GU, lung, hematological cancers	6	**Durham VA Medical Center***	Durham	Ashville, Fayettville, Wilmington, Greenville, Charlotte, Kernersville, Salem, Hampton, Fredericksburg
			Salisbury W.G. (Bill) Hefner VA Medical Center (co-hub)	Salisbury	
			Hunter Holmes McGuire VA Medical Center (co-hub)	Richmond	
	Genitourinary (GU, includes prostate, bladder, kidney), lung	9	**Tennessee Valley Healthcare System**	Nashville	Lexington, Louisville, Memphis, Mountain Home
	Genitourinary (GU, includes prostate, bladder, kidney)	22	**VA Greater Los Angeles Healthcare System***	West Los Angeles	Loma Linda, Long Beach, San Diego, Prescott, Tucson, Phoenix, Albuquerque

5 Sites in remediation^		8	**Jame Haley (Tampa) VA Medical Center***	Tampa	Bay Pines, Miami, Orlando, Gainsville
		7	**Atlanta VA Medical Center**	Decatur	Birmingham, Charleston*, Columbia
		19	**VA Eastern Colorado (Aurora) Healthcare System***	Aurora	Salt Lake City; Muskogee, OK; Oklahoma City, OK; Ft Harrison MT), (Grand Junction, CO; Sheridan WY, Cheyenne WY,)
		17	**VA North Texas Healthcare System**	Dallas	San Antonio, Temple, (Amarillo, Big Spring, Harlingen, El-Paso)
		16	**Central Arkansas Veteran Healthcare System**	Little Rock	Jackson, New Orleans, Shreveport, Alexandria, Biloxi, Fayetteville
			Michael DeBakey VA Medical Center (co-hub)*		

*Denotes POPCaP/GU sites
Co-hub – this site collaborates with the hub sites in leading the VISN
^Remediation – sites addressing gaps and weaknesses found in the plan for developing the cancer clinical research center identified in peer review.